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UNIVERSITY

Education	Harvard University Ph.D. in Population Health Sciences (Health Systems and Economics), 2019–2024 University of Nottingham M.Sc., Economics and Econometrics, Distinction, 2014 University of Nottingham B.Sc., Economics, First Class Honors, 2013		
Fields	Health Economics Industrial Organization Development Economics		
References	Mark Shepard Harvard Kennedy School mark_shepard@hks.harvard.edu	Luca Maini Harvard Medical School maini@hcp.med.harvard.edu	Winnie Yip Harvard T.H. Chan School of Public Health wyip@hsph.harvard.edu
Fellowships & Awards	Fulbright Scholarship, 2019 University of Nottingham Dean's List, 2010–2013; University of Nottingham M.Sc. Scholarship, 2014		
Teaching	Research for Health Economics, Harvard Kennedy School, Teaching Fellow for Prof. Mark Shepard, 2025, 2026 Econometrics for Health Policy, Harvard T.H. Chan School of Public Health, Teaching Fellow for Prof. Terence Cheng, 2022, 2023 Comparative Health Systems I and II, Harvard T.H. Chan School of Public Health, Teaching Fellow for Prof. Winnie Yip, 2022 Introduction to Macroeconomics and Microeconomics, University of Nottingham, Teaching Fellow for Profs. Saumik Paul and Wing Leong Teo, 2012		
Employment	Harvard Kennedy School, Taubman Center for State and Local Government — Postdoctoral Research Fellow, 2026–present Harvard T.H. Chan School of Public Health, Department of Global Health and Population — Postdoctoral Research Fellow, 2024–2026 Harvard T.H. Chan School of Public Health — Visiting Scientist, 2015–2018 World Bank — Consultant (Economic Analyst, International Bank for Reconstruction and Development), 2015, 2019 United Nations Development Programme — Consultant (Program Evaluation, housing projects, Malaysia), 2014		
Research	Research Assistant, University of Nottingham — Professor Saumik Paul, 2013		

Research Assistant, Harvard T.H. Chan School of Public Health — Professor Winnie Yip, 2019–2020

Job Market Paper “Subsidizing Quality for the Poor: Equity and Competition in Health Insurance Markets”

Policymakers often care not just about subsidizing participation but also narrowing quality gaps between the rich and the poor. While subsidies on price *levels* are well understood to raise participation, less is known about *incremental* subsidies that make higher-quality options affordable. This paper studies a progressive incremental subsidy in Massachusetts's health insurance exchange that narrows the price gap between a generous and a basic plan for the poor. An equilibrium model of imperfect competition with vertical differentiation yields three findings. First, the subsidy affords access at the cost of competition: consumers who pay only a share of the price difference are less price-sensitive, and firms raise markups. Second, no standard welfarist planner who weighs consumer surplus against public funds would choose the observed policy. The policy instead reveals a planner with specific egalitarian preferences who values the poor's enrollment in the generous plan directly (Tobin, 1970). Third, an alternate policy of paying consumers a credit for choosing the generous plan yields more access at lower cost. Equalizing prices is thus an inefficient approach to achieving equity within markets.

Publications [“Hospital Allocation and Politicians: Evidence from Malaysia.”](#) World Development, 2026, 207: 107495.

Equitable access to healthcare requires efficient allocation of health infrastructure, yet politics may distort these decisions. This paper studies how politicians distort public hospital allocations in postcolonial Malaysia from 1959 to 2013. Malaysia first allocates hospital funding to districts, then selects specific locations within funded districts for construction. Using six decades of data on hospital construction and elections, I find that constituencies represented by deputy ministers are significantly more likely to receive new hospitals, but only at the location selection stage within districts. District-level funding decisions effectively direct resources to districts far from existing hospitals. I provide suggestive evidence that measurable guidelines for funding decisions exist but not for construction site selection. These results suggest that defining clear guidelines in hospital allocation can limit political distortions.

[“Consumer Choice and Public–Private Providers: The Role of Perceived Prices”](#) (with Wei Aun Yap and Winnie Yip). Health Economics, 2022, 31(9): 1898–1925.

Governments often encourage health service providers to improve quality of care and reduce prices through competition. The efficacy of competition hinges on the assumption that consumers demand high quality care at low prices for any given health condition. In this paper, we examine this assumption by investigating the role of perceived price and quality on consumer choice for four different health conditions across public and private providers. We use a nationally representative survey in Malaysia to elicit respondents' perception on prices and quality, and their preferred choice of provider. We estimate a mixed logit model and show that consumers value different dimensions of quality depending on the health condition. Furthermore, increasing perceived prices for private providers reduces demand for minor, more frequent health conditions such as flu fever or cough, but increases demand for more complex, severe conditions such as coronary artery bypass graft. These findings provide empirical support for price regulation which differentiates the severity of underlying health conditions.

Working Papers “Strategic Auto-Enrollment and Health Insurance Market Design” (with Mark Shepard and Myles Wagner)

Default rules are typically viewed as demand-side tools to steer behavior and increase take-up among passive consumers. We argue that in markets with supply-side competition, defaults also operate as a *market design* policy, shaping firms' incentives to compete on price and quality. We develop a framework for analyzing optimal “strategic defaults” that highlights a central tradeoff

between two effects: a matching effect (assigning passive consumers to plans that fit their needs) and a competition effect (the strength of pricing and product-design incentives induced by the assignment rule). We apply this framework to automatic enrollment in Massachusetts' pre-ACA subsidized health insurance exchange, where roughly one-third of below-poverty enrollees are passively assigned across competing plans with differentiated provider networks. Leveraging quasi-random assignment, we document economically meaningful matching effects: enrollees defaulted into plans covering nearby hospitals use more physician care and are substantially less likely to switch out. We then develop a structural model of active and passive enrollees that quantifies the matching-competition tradeoff and characterizes optimal default rules, including assignments that reward plan quality and exploit consumer-plan match characteristics.

[“Competing Complements in Public–Private Hospital Markets”](#) (Under review, American Economic Journal: Economic Policy)

Public hospitals in many countries provide subsidized care that competes with private hospitals, while training physicians who later join the private sector. I study this tension using the staggered construction of public hospitals in Malaysia from 1996 to 2013. A new public hospital increases private hospital entrants by 36 percent. This crowd-in requires both labor-supply spillovers from public hospitals and districts large enough to support private entry. Reallocating public hospitals to larger districts would raise crowded-in private capacity by roughly a third but reduce equity in access. These findings show public hospitals trade off equity-driven allocation against total healthcare capacity.

[“Does Word-of-Mouth Lead to Better Care? Evidence from Odisha, India”](#) (with Terence C. Cheng and Bijetri Bose) (Under review, Health Economics)

Word-of-mouth (WOM) recommendations from family and friends can substitute for absent quality certification and weak regulatory oversight. Whether such informal information improves or distorts patient decisions remains unclear. We study this question using original linked household and facility survey data from Odisha, India, covering 3,090 patient visits across 373 villages. Patients who choose providers based on recommendations spend more out-of-pocket on drugs and transportation. Yet, this additional spending does not translate into better outcomes: WOM is not associated with higher perceived care quality, and WOM users are significantly less likely to return to the same provider. To understand these patterns, we estimate an attentive logit model that separately identifies consideration and preference stages of provider choice. We find that WOM narrows attention to a smaller set of providers that are not higher-quality. Counterfactual simulations show that removing WOM-induced attention distortions does not significantly improve the quality of providers patients choose, while raising awareness towards other providers yields modest gains. These findings suggest that informal networks cannot reliably substitute for formal quality information systems in fragmented healthcare markets.

Health Policy Papers

McIntyre A, Stone B, **Lim KS**, Shepard M, Sommers BD. “Massachusetts Marketplace Disenrollment Following Enhanced Subsidy Expiration.” *JAMA*, revise and resubmit.

Seminars & Conferences

BU/Harvard/MIT Health Economics Seminar, 2026
Northeast Universities Development Consortium (NEUDC), Tufts University, 2025
American Society of Health Economists (ASHEcon) Annual Conference, 2023–2026

Academic Service

Referee: Social Science & Medicine; Health Systems & Reform

Languages

English (native), Malay (fluent), Mandarin (basic)